

CLINICAL INTELLIGENCE TREND REPORT

YELLOW: Closer Observation Suggested

Patient ID: Wimberley **Period:** December 7 to January 6, 2024 **Report Date:** January 7, 2024 **Coverage:** 478/726h (65.8%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Patient clinical status over 31 days from Dec 07 to Jan 06 ■ HR ■ Breathing



Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
64	77h	Very High Breathing	2	66%

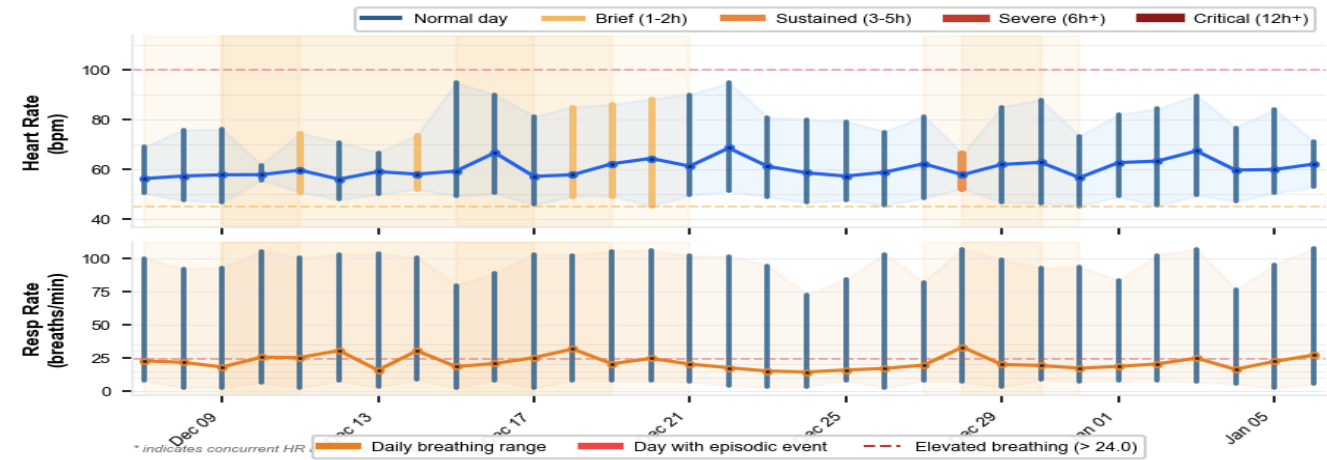
Major Findings: Sustained very high breathing (avg 23, peak 108)

#	Episode	Min/Max	Total Hours	Longest Continuous	Episodes/day	Average	Date
1	Very High Breathing	107 brpm	8h	4h	1	22 brpm	Dec 28 to Dec 31
2	Very High Breathing	106 brpm	29h	2h	2	23 brpm	Dec 07 to Dec 21
3	High Breathing	103 brpm	15h	2h	1	23 brpm	Dec 09 to Dec 17
4	High Breathing	93 brpm	9h	1h	2	23 brpm	Dec 27 to Dec 30
5	Elevated Breathing	61 brpm	2h	1h	<1	23 brpm	Dec 09 to Dec 11
6	Elevated Breathing	95 brpm	3h	1h	<1	23 brpm	Dec 15 to Dec 19

The P5 to P95 heart rate spread was 21 bpm (52 to 73), indicating significant cardiac variability. RR values outside physiologic range (6–60 breaths/min).

Clinical Pattern Observations:

- 1. **High variability:** HR spread 20 bpm (52 to 72).
- 2. **Breathing variability:** RR spread 40 brpm (11 to 52).
- 3. **Clustered pattern:** Episodic events on 6 of 31 days (19%).



Red bars indicate days with detected episodic events.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Clinical Alerting Thresholds

- Very Low HR < 40 bpm
- Low HR < 45 bpm
- Elevated HR > 95 bpm
- High HR > 100 bpm
- Very High HR > 110 bpm
- Elevated Breathing > 24 brpm
- High Breathing > 30 brpm
- Very High Breathing > 40 brpm

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.